10.1 Iron-Deficiency Anemia

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I get tired and breathless climbing stairs." **Objective** • Hb 8.5 g/dL, ferritin low • Conjunctiva pale • HR 104/min on exertion	**ACTUAL** **Activity intolerance** related to reduced oxygen-carrying capacity as evidenced by exertional dyspnea, fatigue and low Hb.	Within 2 weeks of treatment, does her ADL without getting breathless.	• Plan activities with rests in between. • Check HR and SpO_2 while she is active. • Give iron as prescribed; if IV iron is ordered, watch for reactions. • Follow Hb and ferritin.	Copes with ADL: **goal met**.
Subjective • "I do not eat meat often." **Objective** • Eats little iron • Periods are heavy	**ACTUAL** **Deficient knowledge** related to dietary iron and oral iron therapy as evidenced by low-iron diet and questions.	Can name iron-rich foods and explain how to take her iron tablets.	• Teach which foods are rich in iron: red meat, liver, legumes, dark green leaves, dates. • Teach her to take iron with vitamin C, and not with tea, milk or antacids. • Explain that dark stools and constipation are expected. • Refer her to find out why she is losing blood.	Repeats the advice: **goal met**.
Subjective • *No specific complaint* **Objective** • On oral iron • Eats little fibre	**RISK** **Risk for constipation** as evidenced by risk factors: oral iron therapy.	While on iron, soft stools at least every 2–3 days.	• Encourage fluids and fibre. • Chart her bowel pattern. • Give a stool softener if ordered.	Stools regular: **goal met**.